

Meeting Summary — September 4, 2026

Participants: Isak Gutierrez, Thomas Abshier, Joelle Ater, Leonard (CAD/manufacturing) **Duration:** ~4 hours (7:03 AM – 10:57 AM)

Key Decisions & Findings

1. FDA Classification — Lift Is Unavoidable

Following up on the DICE call, the group confirmed: if it lifts, FDA considers it a lift. The discussion shifted from “can we avoid the lift classification” to “what does the lift classification actually require” — and the answer is less scary than expected. FSA and FNG are both 510(k) exempt. The main burden is GMP (quality system), not pre-market authorization.

2. Medicare Is the Big Play

Thomas is firm: Medicare is “the 200-car freight train of sales.” Charlie’s story about Jeanette’s father (complete invalid, got him seated on bed edge, LiftWalker did the rest) convinced Thomas the product qualifies for bed-bound patients — the key Medicare coverage criterion. The group agreed not to close the Medicare door.

3. California Manufacturing Is a Problem

Joelle researched CDPH medical device manufacturing licensing: - \$4,600 application fee (facility-specific, not company-wide, lasts 2 years) - Facility must pass quality control, employee qualifications, manufacturing process, packaging, labeling, documentation reviews - CDPH can investigate complaints, product failures, recalls - Victory Circle would likely not pass as a medical device manufacturer

States with NO separate medical device manufacturing license required: Utah, Minnesota, Michigan, Indiana, North Carolina, Virginia, Wisconsin.

4. Leonard Has Manufacturing Contacts

Leonard joined the call and confirmed multiple options: - **Oshkut** (Spanish Fork, UT) — laser cutting tubing, sheet metal, 8 lasers, online quoting system, tube bending capability coming online - **Suncut Sand** — laser cutting, Leonard has used them before - **Wheelchair manufacturer next door to Oshkut** — doesn’t do contract manufacturing, but proves the ecosystem exists - **Overseas sourcing contact** — a man based in US (grew up in Nigeria), partner in Hong Kong, sources manufacturing across China, Malaysia, Thailand, Vietnam, India. Consolidates under his company name so Ideomotion deals with one vendor - **Previous quotes:** Leonard got quotes from Utah manufacturers before — they were more expensive than Victory Circle, but GMP compliance wasn’t evaluated at the time

Leonard’s action items: 1. Send Traveler tech package to his overseas sourcing contact for a quote 2. Search for local Utah manufacturers along the Wasatch Front 3. LiftWalker (“Flyer”) drawing package expected done next week 4. Also building parts for Thomas’s first brainwave headgear

5. DME Supplier Enrollment — Deep Dive

The group ran a detailed cost analysis on becoming their own DME supplier:

Facility requirements (CMS): - 200 sq ft minimum practice location - Public access (not gated/restricted) - Staffed 30 hours/week minimum - Permanent signage, posted hours - Land-line telephone (no cell as primary) - Must permit unannounced CMS inspections - Cannot share location with another Medicare supplier - Closed-door (no retail showroom) is explicitly allowed - Warehouses don't need separate enrollment

California-specific: If selling Rx devices, must have a licensed exemptee or pharmacist in charge on premises. Other states: anyone can staff the desk.

Costs: - One-time setup: \$9,500–\$25,000 (application fees, accreditation, surety bond) - Annual ongoing: \$50,000–\$78,000 (staff, rent, compliance, accreditation renewal) - Accreditation takes 6–9 months - CMS enrollment processing: 3–6+ months - Total timeline: 12–18 months before first billable claim - Break-even: ~30 units/year sustained

Moratorium update: A nationwide DME enrollment moratorium ran Feb 27 – Aug 27, 2026. It has expired. Enrollment is open again, but CMS can reimpose in 6-month increments.

6. DME Acquisition Option

Two types of listings found on dmelisting.com: - **Shell companies** (\$175K): Brand new, clean PTAN, zero billing history. BUT: under 36 months old, so majority ownership change triggers fresh enrollment — defeats the purpose. - **Operating businesses** (\$300K): e.g., Van Nuys, LA — 15 years operating, licensed, accredited, Medicare + Medi-Cal numbers, 40+ HMO/PPO contracts (some closed to new suppliers).

Key insight: The acquisition target should be an OLD, boring, long-enrolled operator (36+ months, no ownership changes), not a new clean shell. Asset purchases don't transfer enrollment — must be equity deal.

7. Recommendation from Analysis

Do not build or buy a DME operation now. - Not on cost grounds (manageable) — on timing grounds (12–18 months before first claim) - Current volume is near zero; break-even is 30 units/year - 4 other channels need no code: institutions, schools, facilities, cash - Best path: find one enrolled DME dealer willing to carry the product, test the channel, then revisit self-enrollment at 30+ units sustained

Action Items

#	Item	Owner	Status
1	Send Traveler tech package to overseas sourcing contact for quote	Leonard	Open

#	Item	Owner	Status
2	Search for local Utah manufacturers along Wasatch Front	Leonard	Open
3	Complete LiftWalker ("Flyer") drawing package	Leonard	Open (next week)
4	Call Medi-Cal to find out what they actually pay for walkers/lifts	Isak	Open
5	Ask Medi-Cal how a powered lifting walker would be priced under "by report"	Isak	Open
6	Research what type of lawyer needed for medical device sales questions	Joelle	Open
7	Research adult foster care home sales channel (Portland contacts, Cornelia Nitu)	Thomas	Open
8	Research financing options for buyers	Joelle	Open
9	Build deal-killer analysis list for Medicare + DME path	Isak	Open
10	Make a catalog page (required as claims document for Medi-Cal)	Isak	Open
11	Call Petaluma (SciFit) for follow-up	Isak	Open

Open Questions

1. What does Medi-Cal actually pay for powered lifting devices? (Same or similar to walker code = few hundred dollars and dead)
2. Can Victory Circle make parts while another facility does final assembly and is the "manufacturer of record"?
3. Would an out-of-state manufacturer already have GMP compliance?
4. Does selling the existing LiftWalkers without registration create legal problems for future FDA/Medicare operations?
5. Is buying an existing DME (\$250K–\$500K) viable, or is the dealer route sufficient?
6. What are the costs and timeline for the 513(g) formal FDA classification request?
7. Does the HMDR exemptee requirement apply if we sell only to institutions, not individuals?

Key Quotes

Thomas: “The one that turned the corner for me was your dad’s story — he was able to do a completely complete invalid. The only requirement was they were able to get somebody to have their legs dangled off the edge of the bed.”

Thomas: “The big one is the indignity of the bedpan. Being able to say you no longer have to tolerate that — it’s just such a service to people.”

Thomas: “If we can get to that point, we’re rolling, and we’re just trying to catch some airspeed. We’ll eventually be able to pull back the stick.”

Isak: “We’re making up 35 years of research right now.”

Joelle: “This is why it stayed a small mom-and-pop business for almost four decades.”